

MODULE 2

Community First Aid & Basic Life Support

Community First Aid (FA)

Basic Life Support (BLS)

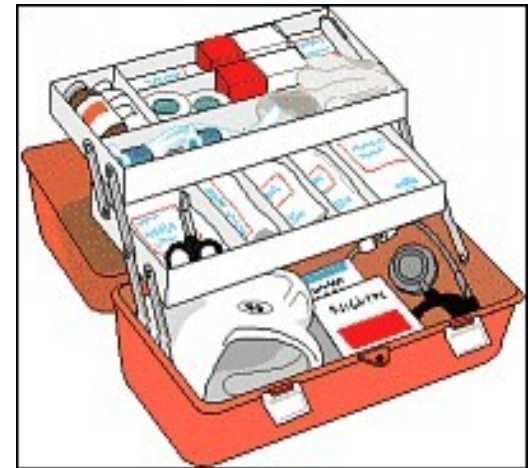


Community First Aid

First Aid

FIRST AID

Is an immediate care given to a person who has been injured or suddenly taken ill. It includes self-help and home care if medical assistance is not available or delayed



First Aid

Roles and Responsibilities of a First Aider

1. Bridge that fills the gap between the victim and the physician

- It is not intended to compete with, or take the place of the services of the physician.
- It ends when the services of a physician begin.

First Aid

Roles and Responsibilities of a First Aider

2. Ensure safety of him / herself and that of bystanders.
3. Gain access to the victim.
4. Determine any threats to patient's life.
5. Summon advanced medical care as needed.
6. Provide needed care for the patient.
7. Assist advanced personnel.
8. Record all findings and care given to the patient.

First Aid

Objectives of First Aid

1. To alleviate suffering
2. To prevent added/further injury or danger
3. To prolong life.



First Aid

Characteristics of A Good First Aider

- | | |
|----------------|--|
| 1. Gentle | - should not |
| cause pain. | |
| 2. Resourceful | - should make the best use of things at hand. |
| 3. Observant | - should notice all signs. |
| 4. Tactful | - should not alarm the victim |
| 5. Emphatic | - should be comforting. |
| 6. Respectable | - should maintain a professional & caring attitude |

First Aid

Hindrances in Giving First Aid

1. Unfavorable surroundings.
2. Presence of crowds.
3. Pressure from victim or relatives.



First Aid

Transmission of Diseases and the First Aider

1. Direct contact
2. Indirect contact
3. Airborne
4. Vector

Body Substance Isolation



First Aid

Basic Precautions and Practices



1. Personal Hygiene



2. Protective Equipment



**3. Equipment Cleaning
& Disinfecting**

First Aid

Suggested First Aid Kit Contents:

- Rubbing alcohol
- Povidone Iodine
- Cotton
- Gauze pads
- Tongue depressor
- Penlight
- Band aid
- Plaster
- Gloves
- Scissors
- Forceps
- Bandage (Triangular)
- Elastic roller bandage
- Occlusive dressing

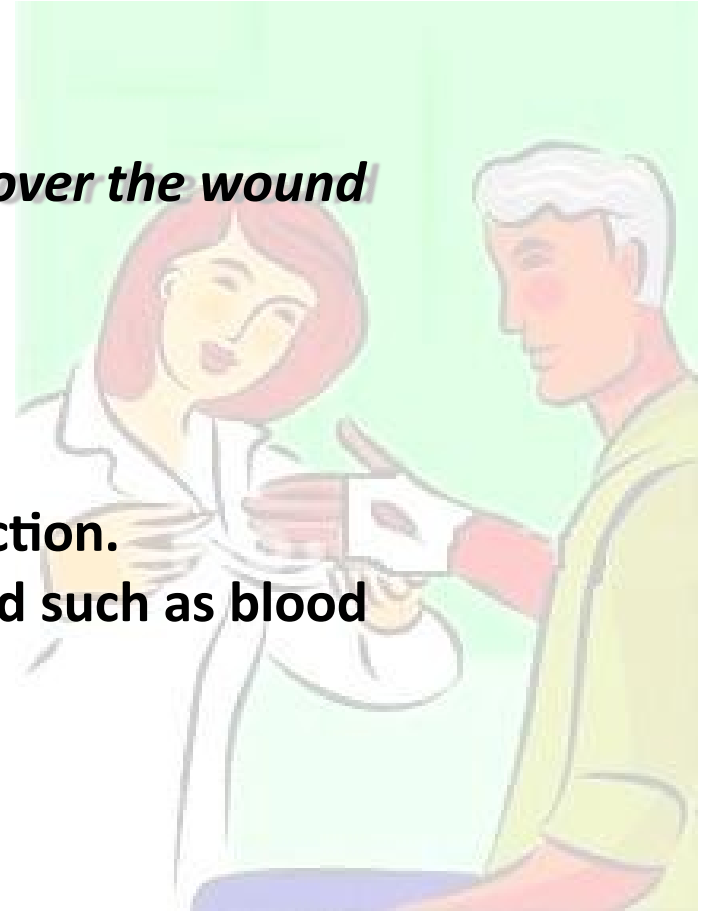
First Aid

DRESSING

Any sterile cloth material used to cover the wound

Other uses of dressing:

- Controls bleeding.
- Protects the wound from infection.
- Absorbs liquid from the wound such as blood plasma, water and pus.



First Aid

BANDAGES

Any clean cloth materials, sterile or not used to hold the dressing in place.



Other uses of bandages:

1. Control bleeding.
2. Tie splints in place.
3. Immobilize body part.
4. For arm support – use as a sling.

Figure 7.

First Aid

GETTING STARTED

1. Plan of Action
2. Gathering of Needed Materials
3. Initial Response:
 - Ask for HELP.
 - Intervene
 - Do no further harm
4. Instruction to Helper/s

First Aid

EMERGENCY ACTION PRINCIPLES

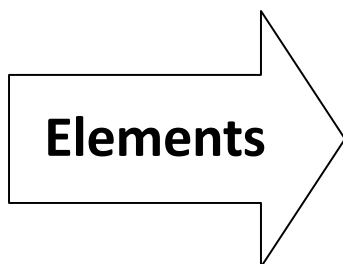
- Survey the Scene



First Aid

EMERGENCY ACTION PRINCIPLES

▪ Survey the Scene



- Is the scene safe?
- What Happened?
- How many people are injured?
- Are there bystanders who can help?
- Identify yourself as a trained First Aider
- Get consent to give care

First Aid

EMERGENCY ACTION PRINCIPLES

▪ **Primary Survey**

- CHECK for CONSCIOUSNESS

- CHECK AIRWAY

- CHECK for Signs of Life

Coughing

Breathing

Movement

First Aid

EMERGENCY ACTION PRINCIPLES

- **Activate Medical Assistance (AMA) or Transfer Facility**

Depending on the situation:

- A bystanders should make the telephone call for help(If available).
- A bystander will be requested to call for a physician.
- Somebody will be asked to arrange for transfer facility.
- Care First or Call First. (Lone Rescuer)

First Aid

EMERGENCY ACTION PRINCIPLES

• **Activate Medical Assistance (AMA) or Transfer Facility**

IF A LONE RESPONDER

- **CALL FIRST** (Activate Medical Assistance before providing care) If:
 - An unconscious adult victim or child 8 years old or older.
 - An unconscious infant or child known to be at a high risk for heart problems.
- **CARE FIRST** (provide first aid for 1-2 minutes and then call fast) If:
 - An unconscious victim less than 8 years old;
 - Cardiac Arrest in children known to be at high risk of arrhythmias
 - Any victim of submersion or near drowning
 - Any victim of arrest associated with trauma
 - Any victim of drug overdose

First Aid

EMERGENCY ACTION PRINCIPLES

- **Activate Medical Assistance (AMA) or Transfer Facility**

Information to be remembered in activating medical assistance :

- What happened?
- Location?
- Number of persons injured?
- Extent of injury and First Aid given?
- The telephone number from where you are calling?
- Person who activated medical assistance must identify him/herself and drop the phone fast.

First Aid

EMERGENCY ACTION PRINCIPLES

▪ Secondary Survey

1. Interview the victim

- Ask victim's name
- Ask what happened
- Assess the SAMPLE History

First Aid

S Signs & symptoms

A Allergies

M Medications

P Past
L medical
history

E Last oral
intake

E Events
prior to the

Always

Always

Always

First Aid

EMERGENCY ACTION PRINCIPLES

▪ Secondary Survey

2. Check vital signs.

- *Pulse Rate*

- *Respiratory Rate*

- *Temperature*

- *Blood Pressure*

- *Skin Appearance*

- *Pupil Reaction*

3. Perform head-to-toe examination.

First Aid

EMERGENCY TRANSFER

Is a rapid movement of patient from unsafe place to a place of safety.

First Aid

EMERGENCY TRANSFER



Danger of fire
or explosion



Danger of toxic gasses or
asphyxia due to lack of
oxygen

First Aid



Serious traffic hazards



Risk of drowning



Danger of electrocution



Danger of collapsing walls

First Aid

TRANSFER

Is moving a patient from one place to another after giving first aid.

Selection of transfer method will depend on the following:

1. Nature and severity of the injury.
2. Size of the victim.
3. Physical capabilities of the first aider.
4. Number of personnel and equipment available.
5. Nature of evacuation route.
6. Distance to be covered.
7. Sex of the victim (Last Consideration).

First Aid

ONE-MAN CARRIES / ASSISTS



Assist to Walk



Carry in Arms

First Aid

TWO-MAN CARRIES



Carry by Extremities



Hand as a litter

First Aid

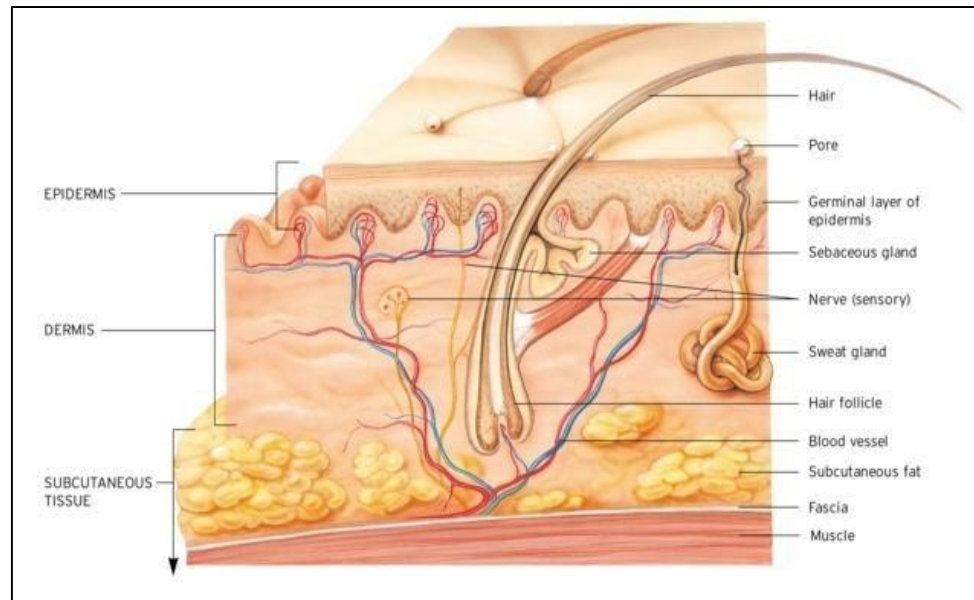
THREE-MAN CARRIES



Hammock Carry

First Aid

Wounds



Always

Always

Always

First Aid

Wounds

Two Types of Wounds

1. *Closed* Closed Wound



First Aid Management

C

S

- Cold Application

S

- Splinting

Always

Always

Always

First Aid

Wounds

Two Types of Wounds

2. *Open Wound*



Puncture



Abrasion



Laceration



Avulsion

Always

Always

Always

First Aid

Wounds

Two Types of Wounds

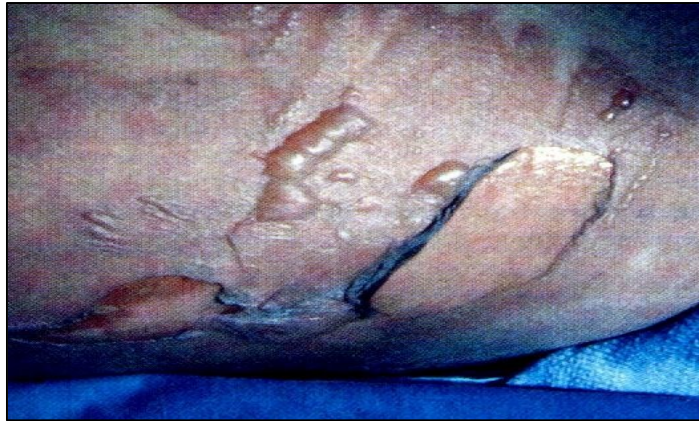
2. *Open* ~~Open~~ Wound

First Aid Management

- C - Control Bleeding
- C - Cover the wound with dressing and secure with a bandage.
- C - Care for shock.
- C - Consult or refer to physician.

First Aid

Burns



Always

Always

Always

First Aid

Burns

TYPES OF BURN INJURIES:



1. Thermal Burns



2. Chemical Burns

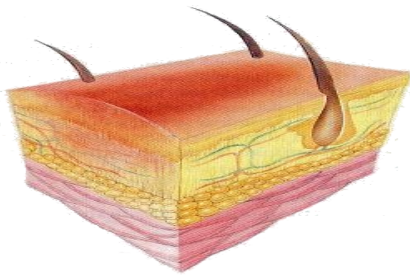


3. Electrical Burns

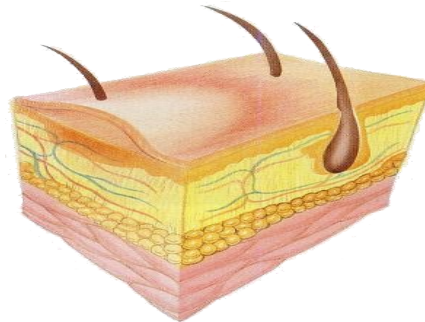
First Aid

Burns

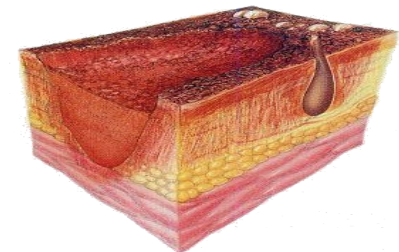
**Superficial
(First-degree) burns**



**Partial-thickness
(Second-degree) burns**



**Full-thickness
(Third-degree) burns**



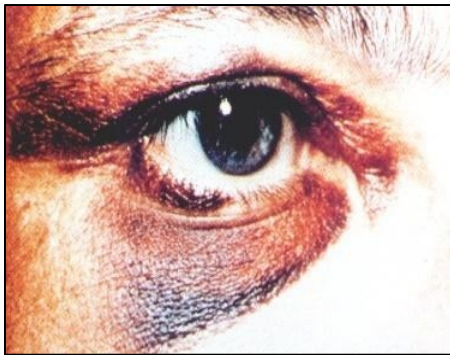
Always

Always

Always

First Aid

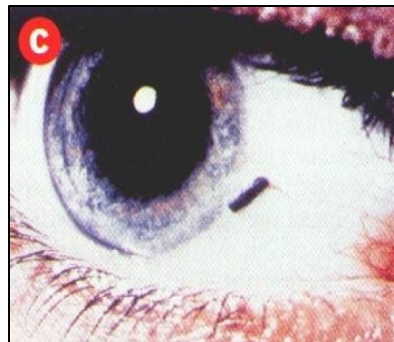
Specific Body Injuries



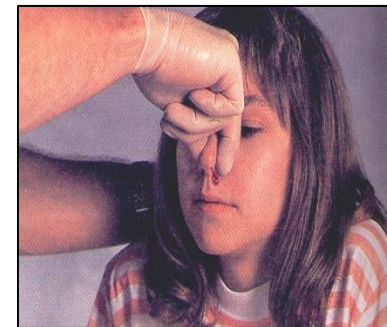
Blows to the Eye



Chemical Burns



Foreign Object



Nose Injuries

First Aid

Specific Body Injuries



4. Impaled Objects



5. Amputations



6. Sucking Chest Wound



7. Abdominal Injuries

Always

Always

Always

First Aid

BANDAGING TECHNIQUES

OPEN PHASE



Head Top Side

First Aid

BANDAGING TECHNIQUES

OPEN PHASE



Chest

Always

Always

Always

First Aid

BANDAGING TECHNIQUES

OPEN PHASE



Burned Hand

First Aid

BANDAGING TECHNIQUES

CRAVAT PHASE



FOREHEAD



ARM/LEG



PALM PRESSURE

Always

Always

Always

First Aid

BANDAGING TECHNIQUES

CRAVAT PHASE



Elbow Bended

Elbow Straight

Always

Always

Always

Dislocation and Broken Bones



FIRST AID MANAGEMENT

- **Check the victims sign of life (if unconscious)**
- **Keep the victim still**
- **If there is bleeding, cover and control bleeding**
- **Immobilize the affected part**
- **Get medical help**

First Aid

BANDAGING TECHNIQUES **FOR IMMOBILIZATION**



Arm Sling



For Sprain

First Aid

FAINTING (Shock)

Signs and Symptoms of Shock

- Face – pale or cyanotic in color.
- Skin – cold and clammy.
- Breathing – irregular.
- Pulse – rapid and weak.
- Nausea and vomiting
- Weakness
- Thirsty

First Aid

First Aid Management:

1. Proper Body Position.



2. Proper Body Heat



3. Proper Transfer

Always

Always

Always

First Aid

INGESTED POISON

Signs and Symptoms

- Altered mental status.
- History of ingesting poisons.
- Burns around the mouth.
- Odd breath odors.
- Nausea, vomiting.
- Abdominal pain.
- Diarrhea

First Aid

INGESTED POISON

First Aid for Ingested Poisoning

1. Try to identify the poison. Call the National Poison Control Center.
2. Place the victim on his or her left side.
3. Monitor ABCs.
4. Save any empty container, spoiled food for analysis.
5. Save any vomitus and keep it with the victim if he or she is taken to an emergency facility.

Heat cramps



FIRST AID MANAGEMENT

- 1. Transfers the victim to a cool place**
- 2. Have the victim rest with his/her feet elevated**
- 3. Cool the victim (do not use an alcohol rub)**
- 4. Give the victim an electrolyte beverages to sip.**
- 5. Massage affected muscle gently and firmly until they relaxes.**

First Aid

Sprain

FIRST AID MANAGEMENT

- 1. Remove all clothing or jewelry around the joint.**
- 2. Apply cold compress at once.**
- 3. Immobilize, Rest and Elevate the affected joint.**
- 4. Seek Medical Help if needed.**



Basic Life Support

Basic Life Support

BASIC LIFE SUPPORT (BLS)



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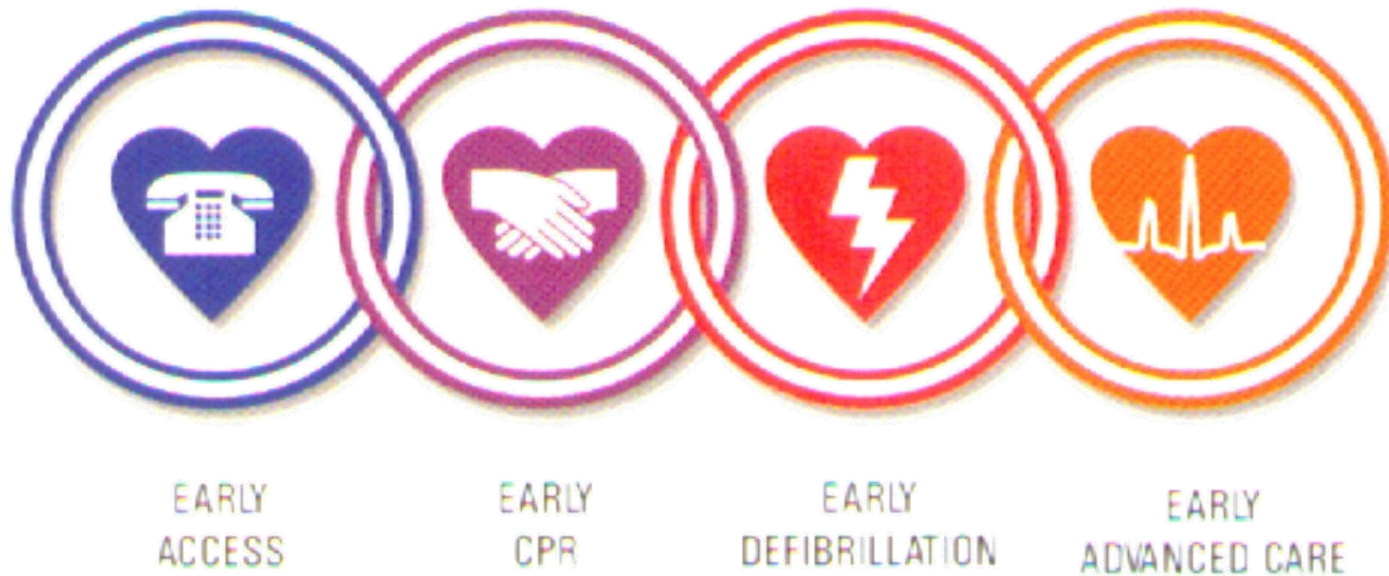
Always

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Basic Life Support

BASIC LIFE SUPPORT (BLS)

Chain of Survival



Basic Life Support

Cardiovascular Disease

RISK FACTORS FOR CARDIOVASCULAR DISEASE

1. Risk factors that cannot be changed (Non-modifiable) :

- Heredity
- Age
- Gender

Basic Life Support

Cardiovascular Disease

RISK FACTORS FOR CARDIOVASCULAR DISEASE

2. Risk factors that can be changed (Modifiable) :

- Cigarette smoking
- Hypertension
- Elevated cholesterol
- Lack of exercises
- Obesity
- Stress
- Diabetes mellitus

Basic Life Support

Cardiovascular Disease

HEART ATTACK (Myocardial Infarction)

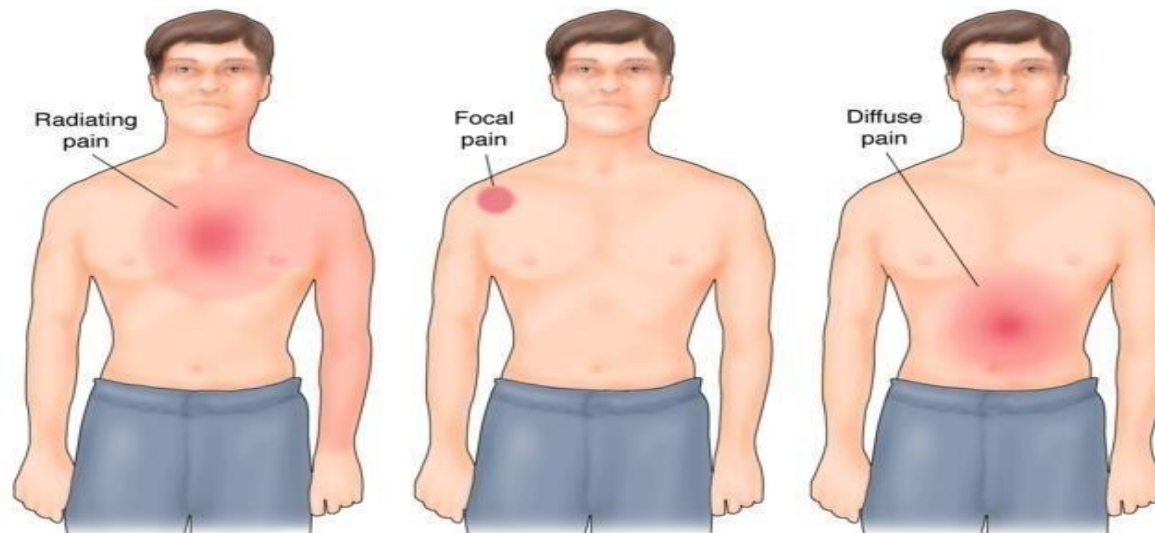
Basic Life Support

Cardiovascular Disease

HEART ATTACK (Myocardial Infarction)

Signs and Symptoms

- Chest Discomfort



Always

Always

Always

Basic Life Support

Cardiovascular Disease

HEART ATTACK (Myocardial Infarction)

Signs and Symptoms

- Sweating
- Nausea
- Shortness of Breathe
- Tingling Sensation

Basic Life Support

Cardiovascular Disease

FIRST AID MANAGEMENT OF HEART ATTACK

1. Recognized the signals of heart attack and take action.
2. Have patient stop what he or she is doing and sit or lie down in a comfortable position. Do not let the patient move around.
3. Have someone call the physician or ambulance for help.

Basic Life Support

Cardiovascular Disease

FIRST AID MANAGEMENT OF HEART ATTACK

4. If patient is under medical care, assist, him/her in taking his/her prescribe medicine/s



Basic Life Support

FOREIGN BODY AIRWAY OBSTRUCTION



COMMON CAUSES:

1. Improper Chewing
2. Excessive Intake of Alcohol
3. Chewing of Dentures
4. Laughing while eating
2. Excessive hand to mouth stage left
unattended
Intake
of
Alcohol

Basic Life Support

FOREIGN BODY AIRWAY OBSTRUCTION

Types of Obstruction:

1. Mild Obstruction - with good air exchange
Mgt: Encourage Coughing
2. Severe Obstruction - with poor or no air exchange
Mgt: Back Blows, Abdominal Thrusts, Chest Thrusts

Basic Life Support

CARDIAC ARREST

Is the condition in which circulation ceases and vital organs are deprived of oxygen.

CARDIOPULMONARY (CPR)

RESUSCITATION

This is a combination of chest compression
and
ventilation.

Basic Life Support

CARDIOPULMONARY RESUSCITATION (CPR)

Criteria for Not Starting CPR

- The patient has a valid “Do Not Attempt Resuscitation” (DNAR) order.
- The patient has signs of irreversible death

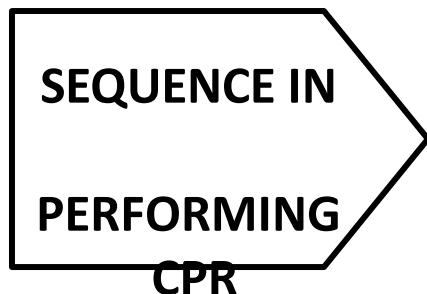
Basic Life Support

CARDIOPULMONARY RESUSCITATION (CPR)

When to **STOP** CPR:

1. **SPONTANEOUS** signs of circulation are restored.
2. **TURNED OVER** to medical services or properly trained and authorized personnel.
3. **OPERATOR** is already exhausted and cannot continue CPR.
4. **PHYSICIAN** assumes responsibility (declares death, take over, etc.).

Basic Life Support



Survey the Scene.

Check Responsiveness

Check Airway (Head-Tilt-Chin Lift)

**Check Signs of Life (Look, Listen & Feel)
for no more than 10 seconds.**

Give 2 Initial breaths

Start CPR if still no signs of life

**Place in Recovery Position if Victim has
signs of life.**

Always

Always

Always

Questions?



THANK YOU!!